

Children's Regional Medical Center — Pediatric Surgery

PEDIATRIC INPATIENT DISCHARGE SUMMARY

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY — CONTAINS NO REAL PATIENT DATA

Patient: Ethan R. Fletcher (Age 11)	DOB: 02/17/2013	MRN: 8834021
Date: October 22, 2024	Provider: Dr. Linda Osei, MD — Pediatric Surgery	

PROCEDURE AND DIAGNOSIS

Diagnosis:	Acute appendicitis, uncomplicated (K37)	Procedure:	Laparoscopic appendectomy (3-port technique)
Date of Surgery:	October 21, 2024	Pathology:	Acute appendicitis without perforation or periappendiceal abscess — pending final
LOS:	1 day (discharged to home with parents)		

HOSPITAL COURSE

Ethan, an 11-year-old male, presented to the pediatric emergency department with 14 hours of periumbilical pain migrating to the right lower quadrant, low-grade fever (38.2°C), nausea, and anorexia. WBC was 14,200 with neutrophilia. CT abdomen/pelvis demonstrated a dilated (9mm), non-compressible appendix with periappendiceal fat stranding, consistent with acute appendicitis. No perforation or abscess was identified.

He was taken to the operating room that evening for laparoscopic appendectomy. The procedure was performed without complication. The appendix was acutely inflamed but non-perforated. He was started on clear liquids postoperatively and advanced to a regular diet by the following morning. Pain was well-controlled with oral acetaminophen and ibuprofen. He ambulated without difficulty and met all discharge criteria by postoperative day 1.

HOME MEDICATIONS

Acetaminophen:	400mg (Children's) PO q6h PRN pain — do not exceed 5 doses/day	Ibuprofen:	300mg PO q8h PRN pain (with food) — alternate with acetaminophen
Augmentin (Amox-Clav):	500/125mg PO twice daily x 3 days — complete full course		

RECOVERY AND ACTIVITY INSTRUCTIONS

Activity: Rest for 2-3 days at home. Light activity (short walks) encouraged after 48 hours. No sports, gym class, swimming, or running for 2 weeks. No lifting over 10 pounds for 2 weeks.

School: May return to school in 3-5 days when pain-free on oral medications and tolerating normal diet. Please bring this note to school nurse.

Diet: Regular diet as tolerated. Start with bland foods if nausea persists. Encourage fluids.

Wound care: Keep the 3 small incision sites clean and dry. Steri-strips will fall off on their own in 7-10 days. Shower is fine after 48 hours; pat dry. No baths or swimming until follow-up.

Return to ER immediately for: fever >38.5°C, increasing abdominal pain or distension, vomiting that prevents oral intake, wound redness/swelling/drainage, or any concerning symptoms. Follow-up with Dr. Osei in 2 weeks.

This document contains entirely synthetic, AI-generated data for demonstration purposes only. No real patient, provider, or institutional information is represented.